

Sick Child: Watch · Call · ER

When to watch at home, call your provider, or go now. **Watch the child, not the number.**

⚠ Under 2 months: ANY fever of 100.4°F (38°C) or higher is an emergency — call or go now, even at night. Do not give fever medicine to a baby this young without your provider's guidance.

SYMPTOM	WATCH AT HOME	CALL PROVIDER	GO TO ER / 911
Fever	Over 2 yr, under 104°F, alert and drinking	Any fever under 2 yr; fever over 3 days; not improving	Under 2 months with any fever; 105°F+; a seizure
Vomiting	Under 24 hr, holding some fluids down, no blood	Over 24 hr in an infant, 48 hr in older child, or repeated in a newborn	Blood in vomit, very lethargic, or signs of dehydration
Diarrhea	Mild, staying hydrated, no blood	More than 5 loose stools a day, or signs of mild dehydration	Blood or mucus in stool, severe dehydration, or infant under 3 months
Croup	Mild barking cough, calm, breathing comfortably	Worsening overnight, or not improved with cool air or steam	Noisy breathing (stridor) at rest, ribs pulling in, bluish lips, or drooling
Ear pain	Mild tugging, no fever, acting okay	Fever with ear pain, or no improvement in 48 hr on antibiotics	Severe pain with high fever, ear drainage, or stiff neck
Rash	Mild, no fever, fades when pressed	Spreading, hives with mild swelling, or a rash after fever breaks	Does NOT fade when pressed — ER now; facial/lip/throat swelling — 911
Breathing	Mild congestion, breathing normally at rest	Fast or noisy breathing (not croup), or persistent wheezing	Ribs pulling in, blue lips, grunting, or can't cry/speak

Call 911 or go to the ER right now if:

Won't wake or is very hard to rouse · bluish lips or fingernails · difficulty or fast breathing at rest · stiff neck, bad headache, or light sensitivity · a rash that does NOT fade when pressed · a seizure of any kind (even a first one).

Dehydration — act quickly: no wet diaper in 8 hr · no tears when crying · dry, cracked lips · sunken eyes or soft spot · extremely sleepy or hard to wake.

Never give aspirin to a child (risk of Reye's syndrome). Treat for comfort — dress lightly, offer fluids often, rest — not to chase a number.

Weight-Based Medication Guide

Dose by your child's **current weight**, not age alone. Confirm the concentration on your bottle before every dose.

 **Acetaminophen (Tylenol)** — every 4–6 hours · from 2 months if fever · max 5 doses in 24 hr

WEIGHT	DOSE — LIQUID 160 MG / 5 ML
6–11 lbs 2.7–5 kg	1.25 mL
12–17 lbs 5.5–7.7 kg	2.5 mL
18–23 lbs 8–10.5 kg	3.75 mL
24–35 lbs 11–15.9 kg	5 mL
36–47 lbs 16–21 kg	7.5 mL
48–59 lbs 22–26 kg	10 mL
60–71 lbs 27–32 kg	12.5 mL
72–83 lbs 33–37 kg	15 mL
84–95 lbs 38–43 kg	17.5 mL

Infant drops & Children's liquid are the **same concentration** (160 mg / 5 mL) — confirm the label. **Do NOT** exceed 5 doses in 24 hr · **Do NOT** use for pain only under 2 months without provider guidance.

 **Ibuprofen (Motrin / Advil)** — every 6–8 hours · 6 months+ · with food · max 4 doses in 24 hr

WEIGHT	INFANT DROPS (50 MG / 1.25 ML)	CHILDREN'S (100 MG / 5 ML)
18–23 lbs 8–10.5 kg	1.25 mL	—
24–35 lbs 11–15.9 kg	1.875 mL	5 mL
36–47 lbs 16–21 kg	—	7.5 mL
48–59 lbs 22–26 kg	—	10 mL
60–71 lbs 27–32 kg	—	12.5 mL
72–83 lbs 33–37 kg	—	15 mL
84–95 lbs 38–43 kg	—	17.5 mL

Not safe for infants under 6 months · always give with food or milk · **Do NOT** give if vomiting or dehydrated. Infant drops and Children's liquid are **different** concentrations — check the label.

Never give aspirin to a child. Do not alternate Tylenol and Motrin on a set schedule unless your provider directs you to.

Sick-Day Kit & Quick Info

Build it before you need it — so you're not searching at 2 AM.

Medicine & supplies

- ☐ Acetaminophen (Tylenol); ibuprofen if 6 months+
- ☐ Oral rehydration solution (Pedialyte) — keep one on hand
- ☐ Digital thermometer (rectal for infants)
- ☐ Nasal saline drops & bulb syringe / NoseFrida
- ☐ Antihistamine (ask your provider which)
- ☐ Hydrocortisone 1% cream; petroleum jelly
- ☐ Cool-mist humidifier (cleaned weekly)
- ☐ Medicine syringe or dropper; check expiry every 6 mo

Feeding during illness

- ☐ Breastfeed on demand — continue through illness
- ☐ Oral rehydration in small, frequent sips
- ☐ BRAT foods: banana, rice, applesauce, toast
- ☐ Avoid juice and sports drinks for young infants
- ☐ Popsicles for hydration and sore throats
- ☐ Back to normal food after 6 hr with no vomiting

Keep home from daycare / school until...

- ☐ Fever-free 24 hr without medicine
- ☐ 24 hr after the last vomiting or diarrhea
- ☐ 24 hr on antibiotics for strep
- ☐ Pink eye with discharge — until treatment has started

Emergency numbers — fill in

Pediatrician — main line

Pediatrician — after-hours

Nearest children's ER

Urgent care (kids)

Poison Control — 1-800-222-1222

Know your child's info

Current weight (needed for dosing)

Allergies — medicines & foods

Current medicines & doses

Blood type (if known)

Insurance / Medicaid ID — photo saved to phone

Pediatrician's fax (for records)

When in doubt, call your pediatrician — no question is too small. Trust your gut. You know your child.

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A fever is 100.4°F (38°C) or higher

SYMPTOM	WATCH AT HOME	CALL PROVIDER	GO TO ER / 911
Fever	Alert, drinking, under 104°F	Over 104°F, or lasting more than 3 days, or not improving	Stiff neck, won't wake, confused, or a seizure
Sore throat	Mild, with cold symptoms (cough, runny nose) — usually viral	Fever, swollen glands, or no cold symptoms — may need a strep test	Drooling, can't swallow, muffled voice, or trouble breathing
Vomiting / diarrhea	Keeping some fluids down, no blood, under 24 hr	Over 24–48 hr, can't keep fluids down, or signs of dehydration	Blood in vomit or stool, severe belly pain, or very lethargic
Abdominal pain	Mild, comes and goes, still eating and passing gas	Persistent, with fever, or not improving	Severe — especially lower right — with vomiting/fever, or a rigid belly
Breathing / asthma	Mild congestion, breathing normally at rest	Persistent cough or wheeze, fast breathing, not better with inhaler	Can't speak in full sentences, ribs pulling in, blue lips, severe wheeze
Headache	Mild, eases with rest, fluids, or OTC medicine	Frequent, with fever, or interfering with school	Sudden “worst ever,” with stiff neck, confusion, vision change, or after a head injury
Rash	Mild, no fever, fades when pressed	Spreading, with fever, or itchy hives with mild swelling	Does NOT fade when pressed — or facial/lip/throat swelling or trouble breathing

Call 911 or go to the ER right now if:

Trouble breathing or fast breathing at rest · bluish lips or fingernails · a seizure of any kind (even a first one) · won't wake, very confused, or unresponsive · stiff neck with fever · a rash that does NOT fade when pressed.

Dehydration — act quickly: no urination in 8+ hours · dark urine · dizzy or lightheaded · dry, cracked lips · no tears · very sleepy or hard to wake.

Never give aspirin to a child or teen (risk of Reye's syndrome). Comfort care: offer fluids often, encourage rest, dress lightly — treat for comfort, not to chase a number.

Fever & Pain Medication Guide

Dose by your child's **current weight**, not age alone. Confirm the concentration on your bottle before every dose.

Keep your child's current weight handy — every dose is based on it

Acetaminophen (Tylenol) — every 4–6 hours · max 5 doses in 24 hr

AGE / WEIGHT	DOSE	CHILDREN'S LIQUID (160 MG / 5 ML)	ADULT TABLET (325 MG)
6–11 yr (48–95 lbs)	320–480 mg	10–15 mL	1 tablet
12+ yr / adult (96+ lbs)	480–1000 mg	15 mL or tablet form	1–2 tablets

Do NOT exceed 5 doses in 24 hours · **Do NOT** combine with other products containing acetaminophen · **Do NOT** use in a child with liver disease without provider guidance.

Ibuprofen (Advil / Motrin) — every 6–8 hours · max 4 doses in 24 hr · give with food

AGE / WEIGHT	DOSE	CHILDREN'S LIQUID (100 MG / 5 ML)	ADULT TABLET (200 MG)
6–11 yr (48–77 lbs)	200–300 mg	10–15 mL	1 tablet
12+ yr (78+ lbs)	400 mg	20 mL	2 tablets

Do NOT give if dehydrated, vomiting, or with known kidney problems · **Do NOT** combine with other NSAIDs · always give with food or milk.

Never give aspirin to a child or teen. Do not alternate Tylenol and Motrin on a set schedule unless your provider directs you to.

When in doubt, call your provider — no question is too small. Always use the measuring device that comes with the medicine, not a kitchen spoon.

Sick-Day Kit & Quick Info

Build it before you need it — so you're not searching at 2 AM.

Medicine & supplies

- ☐ Acetaminophen (Tylenol) & ibuprofen (Advil/Motrin)
- ☐ Oral rehydration / electrolyte drink
- ☐ Digital thermometer (oral or temporal)
- ☐ Antihistamine (ask your provider which)
- ☐ Hydrocortisone 1% cream for mild rash/itch
- ☐ Throat lozenges / saline spray (age-appropriate)
- ☐ Medicine syringe or dosing cup
- ☐ Check expiration dates every 6 months

Comfort & food during illness

- ☐ Small, frequent sips of fluids or electrolytes
- ☐ Bland foods: toast, crackers, rice, applesauce, broth
- ☐ Popsicles for hydration and sore throats
- ☐ Rest; ease back to normal food after 6 hr with no vomiting

Keep home from school until...

- ☐ Fever-free 24 hr without medicine
- ☐ 24 hr after the last vomiting or diarrhea
- ☐ 24 hr on antibiotics for strep
- ☐ Pink eye with discharge — until treatment has started

Emergency numbers — fill in

Primary care / clinic

After-hours / nurse line

Nearest ER

Urgent care

Pharmacy

Poison Control — 1-800-222-1222

Know your child's info

Current weight (for dosing)

Allergies — medicines & foods

Current medicines & doses

Chronic conditions (asthma, etc.)

Insurance / Medicaid ID

Emotional health is health, too. A move or a hard season can show up as withdrawal, anger, changes in sleep or appetite, or stomachaches and headaches with no clear cause. If you're worried about a child's mood — or their safety — call your provider, or reach the **988 Suicide & Crisis Lifeline** any time by calling or texting **988**. When in doubt, ask. You know your child.